

# eze<sup>®</sup>-D

## Oral Solution

### COMPOSITION

Each 5ml contains Loratadine 5mg and Pseudoephedrine Sulfate 60mg.  
Sodium Benzoate 0.1% w/v as preservative.

### PHARMACODYNAMICS

Loratadine is a long-acting, non-sedating H<sub>1</sub>-antagonist antihistamine with no significant antimuscarinic activity.

Pseudoephedrine is a direct and indirect acting sympathomimetic. It is a stereoisomer of ephedrine and has a similar action, but has been stated to have less pressor activity and fewer CNS effects.

### PHARMACOKINETICS

Loratadine is rapidly absorbed from the gastrointestinal tract after oral doses, peak plasma concentrations being attained in about 1 hour. Bioavailability is increased and time to peak plasma concentrations is delayed when taken with food.

Loratadine undergoes extensive metabolism. The major metabolite, desloratadine has potent antihistaminic activity. Reported mean elimination half-lives for loratadine and desloratadine are 8.4 and 28 hours respectively.

Loratadine is about 98% bound to plasma proteins and desloratadine is less extensively bound. Most of the dose is excreted equally in the urine and faeces, mainly in the form of metabolites.

Pseudoephedrine is readily absorbed from the gastrointestinal tract. It is excreted largely unchanged in the urine with small amounts of its hepatic metabolite. It has a half-life of about 5 to 8 hours; elimination is enhanced and half-life accordingly shorter in acid urine.

### INDICATIONS

eze<sup>®</sup>-D is indicated for the relief of symptoms associated with allergic rhinitis and the common cold including nasal congestion, sneezing, rhinorrhea, pruritus and lacrimation. eze<sup>®</sup>-D is recommended when both the antihistaminic properties of loratadine and the decongestant effect of pseudoephedrine sulfate are desired.

## **DOSAGE AND ADMINISTRATION**

For oral use only. Can be taken with or without food.

### Adults

5ml (1 teaspoon) twice daily.

### Children 6 -12 years old

- Body weight > 30kg : 5ml (1 teaspoon) twice daily.
- Body weight ≤ 30kg : 2.5ml (½ teaspoon) twice daily.

## **CONTRAINDICATIONS**

Known hypersensitivity to Loratadine and/or Pseudoephedrine Sulfate. Also contraindicated in patients receiving MAO inhibitor therapy or within fourteen days of discontinuing such treatment and in patients with narrow angle glaucoma, urinary retention, severe hypertension, severe coronary artery disease and hyperthyroidism.

## **WARNINGS AND PRECAUTIONS**

Sympathomimetics should be used with caution in patients with glaucoma, stenosing peptic ulcer, pyloroduodenal obstruction, prostatic hyperthrophy or bladder neck obstruction, cardiovascular disease, increased intraocular pressure or diabetes mellitus.

Sympathomimetics should be used with caution in patients receiving digitalis. Sympathomimetics may cause central nervous system (CNS) stimulation, excitability, convulsions, and/or cardiovascular collapse with accompanying hypotension. Pseudoephedrine sulfate may cause excitability, especially in children.

In patients 60 years of age and older, sympathomimetics are also more likely to cause adverse reactions such as confusion, hallucination, convulsions, CNS depression and death.

Patients with severe liver impairment should be administered a lower initial dose because they may have reduced clearance of loratadine; an initial dose of one-half the usual dose is recommended.

Drug Abuse and Dependence: There are no data available to indicate that abuse or dependency occurs with Loratadine. Pseudoephedrine Sulfate, like other CNS stimulants, has been abused. At high doses, subjects commonly experience an elevation of mood, decreased appetite and a sense of increased physical energy, mental capacity and alertness. Anxiety, irritability and loquacity also have been experienced. With continued use, tolerance develops; the user increases the dose and ultimately toxicity occurs. Depression may follow rapid withdrawal.

Pediatric Usage: Safety and efficacy of ezedex-D in children younger than 6 years of age have not yet been established.

**Warning**

When used for treatment cough and cold:

- a) Not to be used in children less than 2 years of age
- b) To be used with caution and doctor's/pharmacist's advice in children 2 to 6 years of age.

**DRUG INTERACTIONS**

Loratadine is metabolized by cytochrome P450 isoenzymes CYP3A4 and CYP2D6. Therefore, use with other drugs that inhibit or are metabolized by these hepatic enzymes may result in changes in plasma concentrations of either drug, or possibly adverse effects. Drugs known to inhibit one or other of these enzymes include cimetidine, erythromycin, ketoconazole, quinidine, fluconazole and fluoxetine.

When sympathomimetics are given to patients receiving monoamine oxidase (MAO) inhibitors, hypertensive reactions, including hypertensive crises may occur.

The antihypertensive effects of methyldopa, mecamylamine, reserpine and veratum alkaloids may be reduced by sympathomimetics. Beta-adrenergic blocking agents may also interact with sympathomimetics. Increased ectopic pacemaker activity can occur when pseudoephedrine is used concomitantly with digitalis. Antacids increase the rate of pseudoephedrine absorption; kaolin decreases it.

**PREGNANCY AND LACTATION**Pregnancy

To be avoided during pregnancy due to the severity of the abnormality in newborns and the availability of alternatives to pseudoephedrine.

Lactation

Since Loratadine and Pseudoephedrine Sulfate are excreted in breast milk, a decision should be made whether to discontinue nursing or discontinue the use of this product.

**SIDE EFFECTS**

As an antihistamine, Loratadine occasionally causes gastrointestinal disturbances include nausea, vomiting, diarrhoea or epigastric pain; palpitations and arrhythmias. It may cause rashes and hypersensitivity reactions (including bronchospasm, angioedema and anaphylaxis). Blood disorders, including agranulocytosis, leucopenia, haemolytic anaemia, and thrombocytopenia, although rare, have been reported. Other adverse effects that have been reported with the antihistamines include convulsions, sweating, myalgia, paraesthesia, extrapyramidal effects, tremor, sleep disturbances, depression, confusion, tinnitus, hypotension and hair loss.

As a sympathomimetic, Pseudoephedrine Sulfate may cause central effects include anxiety, fear, restlessness, insomnia, confusion, irritability, headache and psychotic states; dyspnoea, weakness, anorexia, nausea and vomiting are also common. Palpitations, tachycardia, arrhythmias, hypertension, hypotension, with dizziness, fainting and flushing

are reported. Other effects include gangrene, mydriasis, difficulty in micturition and urinary retention, piloerection, sweating, increased salivation, hypokalaemia, muscle tremor and hyperglycaemia.

The commonest adverse effects of pseudoephedrine include tachycardia, anxiety, restlessness and insomnia. Skin rashes and urinary retention have occasionally occurred. Hallucinations have been reported rarely, particularly in children.

### **SYMPTOMS AND TREATMENT FOR OVERDOSAGE**

In the event of overdosage, general symptomatic and supportive treatment should be started immediately and maintained for as long as necessary.

Symptoms may vary from CNS depression (sedation, apnea, diminished mental alertness, cyanosis, coma, cardiovascular collapse) to stimulation (insomnia, hallucination, tremors or convulsions) to death. Other signs and symptoms may be euphoria, excitement, tachycardia, palpitations, thirst, perspiration, nausea, dizziness, tinnitus, ataxia, blurred vision and hypertension or hypotension. Stimulation is particularly likely in children, as are atropine-like signs and symptoms (dry mouth; fixed, dilated pupils; flushing; hyperthermia; and gastrointestinal symptoms).

In large doses sympathomimetics may give rise to giddiness, headache, nausea, vomiting, sweating, thirst, tachycardia, precordial pain, palpitations, difficulty in micturition, muscular weakness and tenseness, anxiety, restlessness and insomnia. Many patients can present a toxic psychosis with delusions and hallucinations. Some may develop cardiac arrhythmias, circulatory collapse, convulsions, coma and respiratory failure.

The patient should be induced to vomit, even if emesis has occurred spontaneously. Pharmacologically induced vomiting by the administration of ipecac syrup is a preferred method. However, vomiting should not be induced in patients with impaired consciousness. The action of ipecac is facilitated by physical activity and by the administration of 240 to 360 millilitres of water. If emesis does not occur within 15 minutes, the dose of ipecac should be repeated. Precautions against aspiration must be taken, especially in children.

Following emesis, adsorption of any drugs remaining in the stomach may be attempted by the administration of activated charcoal as a slurry with water. If vomiting is unsuccessful, or contraindicated, gastric lavage should be performed. Physiologic saline solution is the lavage solution of choice, particularly in children. In adults, tap water can be used; however, as much as possible of the amount administered should be removed before the next instillation. Saline cathartics draw water into the bowel by osmosis and therefore may be valuable for their action in rapid dilution of bowel content. It is not known whether this product is dialyzable. After emergency treatment, the patients should continue to be medically monitored.

Treatment of signs and symptoms of overdosage is symptomatic and supportive. Stimulants (analeptic agents) should not be used. Vasopressors may be used to treat hypotension.

Short-acting barbiturates, diazepam or paraldehyde may be administered to control seizures. Hyperpyrexia, especially in children, may require treatment with tepid water sponge baths or hypothermic blanket. Apnea is treated with ventilatory support.

#### **EFFECTS ON ABILITY TO DRIVE AND USE MACHINE**

Although drowsiness is rare, nevertheless patients should be advised that it can occur and may affect performance of skilled tasks such as driving.

#### **STORAGE CONDITION**

Store below 30°C.

#### **SHELF LIFE**

The expiry date is indicated on the packaging.

#### **PRODUCT DESCRIPTION, DOSAGE FORM AND PACKAGING AVAILABLE**

Clear, yellowish oral solution with grape flavour.

Available in pack sizes of 60ml, 90ml and 120ml.

For further information, please consult your physician or pharmacist.

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Manufacturer and Product Registration Holder

**Xepa-Soul Pattinson (Malaysia) Sdn Bhd**

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